

Chapter 1

The Six-Pillar Framework

The Health Reform Portfolio — What the Six Pillars Are, How They Relate, and Why All Must Move Together

Chapter 1 establishes the six-pillar framework that organizes this entire book and to which every subsequent chapter returns. It answers three questions: what each pillar is and what function it performs in the transformation system; what it means to think of those six pillars as a portfolio rather than a checklist; and what the reader will find in each pillar's dedicated chapter. The full architecture of dependency relationships between the pillars — all fifteen directed connections — is developed in Chapter 1A. This chapter provides the foundation that makes Chapter 1A legible.

I. This Is a Portfolio, Not a Plan

The Health Reform Portfolio: Six Pillars, Proven Sequence, Transferable Framework. That title makes a specific claim about how healthcare transformation should be understood and managed — a claim this chapter unpacks before any pillar is defined.

A plan assumes a defined end state and a linear path to reaching it. It assumes that the policy environment, the financial context, the technological landscape, and the organizational capabilities of the implementing institutions will remain stable enough that the document written today will accurately describe what happens over the next five years. In healthcare transformation, none of these assumptions hold. Federal policy changes mid-reform. Hospital systems consolidate or fail. Technology evolves faster than governance can follow. Workforce shortages worsen unexpectedly. Political coalitions shift. A plan written for a stable system is an artifact of wishful thinking in a dynamic one.

A portfolio is different. A portfolio is a set of active investments, managed together, with explicit attention to how they interact, how their risks correlate, what their sequencing dependencies are, and how the whole must be adjusted when any single element underperforms or the environment shifts. Portfolio management does not commit to a single predetermined path. It commits to a direction, a set of foundational investments that must be made regardless of how the environment shifts, and a decision logic for adaptation.

The six-pillar framework is the architecture of that portfolio. Policy, Technology, Economics, Clinical, Equity, and Operations are its six asset classes. The sequencing logic developed in Chapter 2 is its investment horizon framework. The dependency analysis in Chapter 1A is its risk management architecture. Vermont is the portfolio's most detailed case study — what it looks like to manage all six asset classes simultaneously in a real system under real financial pressure, without the luxury of a blank canvas.

What a Portfolio Approach Requires That a Plan Does Not

Explicit acknowledgment of uncertainty. A portfolio manager does not pretend to know exactly how each investment will perform. The portfolio is structured to produce acceptable outcomes across a range of scenarios, not optimal outcomes in the single scenario the planner expects.

Active management of interdependencies. Portfolio assets interact. A technology investment that fails affects the economics investment that depended on it. A policy reversal at the federal level affects every pillar simultaneously. These interactions must be monitored and managed in real time, not assumed away in the planning document.

Explicit sequencing discipline. Not all investments can be made simultaneously with equal effect. Some must precede others — not because the planner prefers that order, but because the later investment cannot function without the earlier one already in place. Portfolio management makes sequencing a design constraint, not a scheduling preference.

Tolerance for partial failure. Not every pillar investment will perform as designed. The portfolio approach treats pillar underperformance as an informative signal requiring adjustment, not a catastrophic deviation from a plan that must be concealed or explained away.

A standard of success measured across the whole portfolio. Individual pillar success that does not produce system-level transformation is not success. The standard is population health improvement, affordability, and equity — outcomes that the portfolio as a whole must produce, not outcomes that any single pillar can claim independently.

II. The Failure That Makes the Framework Necessary

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued.

The pattern of failure across those models is analytically consistent — and it is the failure the six-pillar framework is designed to prevent. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar produced quality improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment found that providers lacked the operational capacity to use the tools effectively.

This is the defining failure mode of American healthcare reform: single-pillar thinking. Addressing payment reform, or clinical redesign, or technology investment — one at a time, in isolation — while leaving the other pillars unchanged. Each isolated intervention runs directly into the constraints created by the pillars it ignored. The result is technically correct recommendations that collapse on contact with a system that was not redesigned to support them.

"The problem is not the list of what needs to change. Every serious framework for the past thirty years has identified essentially the same list. The problem is treating it as a list — a set of independent items to be checked in any order — rather than as a system of interdependent investments to be sequenced, managed, and held accountable together."— The Health Reform Portfolio

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. The six pillars of the framework are not a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. The checklist framing is precisely how decades of technically correct reform recommendations have produced inadequate results.

III. The Six Pillars Defined

The six pillars are not equal in their causal priority — Policy and Technology are enabling preconditions that others depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional. Remove any one and the others are undermined. The full dependency architecture is developed in Chapter 1A. What follows here is the definition of each pillar: what it is, what function it performs in the transformation system, and how Vermont has applied it.

Pillar	Definition, Structural Role, and Vermont Application
Policy	The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. It performs two functions no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital — federal grant awards, federal-state agreements — that funds the technology build. Voluntary reform operates within the Policy pillar's existing constraints; structural transformation requires those constraints to change. <i>Vermont application: Acts 167 (2022), 51 (2023), and 68 (2025) are Vermont's Policy pillar reform cascade. Each converts voluntary reform into statutory obligation. The AHEAD Model State Agreement (January 2025) and the \$195M Rural Health Transformation Program award are Policy pillar products — the federal relationships and capital that make the technology build financially feasible. The Green Mountain Care Board's mandatory rate-setting authority is the institutional Policy pillar asset that makes mandatory payment reform operationally executable.</i>
Technology	The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. Technology is the information substrate on which all other pillar management functions depend — you cannot manage what you cannot measure. It must be in place before financial risk is assumed under a value-based payment model. The OneCare Vermont failure — managing global budget accountability without integrated, timely data — is the canonical proof of what happens when Economics precedes Technology in the execution sequence. <i>Vermont application: VHCURES (all-payer claims database), VTIL/VHIE (health information exchange), the AHS-GMCB analytics vendor procured in early 2026, AI scribe and remote patient monitoring grants through the RHT Program, and Vermont's Clinically Integrated Network are Vermont's Technology pillar investments. The two-year data lag in current VHCURES reporting is the Technology pillar's most consequential unresolved gap as AHEAD's global budgets approach their January 2027 launch.</i>
Economics	The payment and financial architecture that determines what behaviors the healthcare system rewards. The Economics pillar encompasses payment reform from fee-for-service to value-based and global payment, financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time. This pillar has a critical internal distinction: Economics-as-design (defining the payment architecture — can happen in parallel with the Technology build) versus Economics-as-management (running global budgets operationally — requires Technology to already be functional). <i>Vermont application: Mandatory reference-based pricing beginning FY2027, global hospital budgets beginning FY2028, the AHEAD Model's total cost of care accountability, Act 68's hospital spending reduction requirement (2.5% in FY2026), and the GMCB's FY2026 commercial rate benchmark (-1%)</i>

	are Vermont's Economics pillar instruments. The AHEAD EAST Fund (up to \$150M annually) is Economics pillar transformation capital — distinct from the incentive architecture, but equally essential.
Clinical	The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes — or fails to become — patient outcomes. Clinical redesign without payment alignment reverts when the incentives revert: this is among the most empirically robust findings from a decade of value-based payment research. Clinical transformation comes fourth in the execution sequence because it requires aligned payment incentives (Stage 3) and data infrastructure (Stage 2) to already be in place before population health management can function as designed. <i>Vermont application: The Blueprint for Health (all-payer PCMH program, 5.8:1 documented ROI), the Collaborative Care Model for behavioral health integration in primary care, CCBHC expansion (seven certified entities by mid-2026), the primary care investment strategy under AHEAD, Hub-and-Spoke SUD treatment, PACE program development, and the Centers of Excellence regionalization framework are Vermont's Clinical pillar investments.</i>
Equity	The programs, investments, accountability structures, and payment design requirements that ensure transformation benefits reach all populations — and specifically those most harmed by the current system's failures. Equity is not a soft afterthought or a supplemental program layered on top of the transformation; it is an analytical requirement and a constraining dependency on every other pillar's design. A transformation that reduces average costs while widening geographic or demographic disparities has failed by the framework's own standards. Critically, equity must constrain payment design at Stage 3 — through social risk adjustment in global budgets — not merely be measured at Stage 5. <i>Vermont application: Geographic equity across Vermont's 14 Hospital Service Areas (with particular focus on the Northeast Kingdom's 6-8% uninsurance rate against the 3% statewide average), SDOH investment through the AHEAD EAST Fund and Blueprint HRSN universal screening, rural access preservation requirements in RBP and global budget design, the Rural Emergency Hospital model, health equity metrics embedded in Act 167/68 monthly reporting, and the Health Equity Advisory Commission's role in the Statewide Strategic Plan are Vermont's Equity pillar investments and accountability structures.</i>
Operations	The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents — the graveyard of technically correct reforms that lacked the organizational infrastructure to implement them. Operations is sixth in the execution sequence not because it is least important, but because operational redesign must be calibrated to the payment environment it is executing within. A hospital that redesigns its administrative operations for fee-for-service before its payment model is live optimizes for the world that transformation is replacing. <i>Vermont application: The 14-hospital transformation planning process (148 operational commitments across Vermont hospitals as of January 2026), the Rural Health Redesign Center technical assistance methodology, the AHS HSA Coordinator model, the proposed Division of Planning and Effectiveness, EMS regionalization, administrative simplification of CON and prior authorization processes, and the Vermont CIN's shared services architecture are Vermont's Operations pillar investments. Vermont's critical-access hospitals spend \$2,730 per adjusted discharge on administration against a \$1,427 national benchmark — the quantified opportunity the Operations pillar must capture.</i>

Figure 1.1 — The Six Pillars: definitions, structural roles, and Vermont applications. Source: six-pillar framework documentation; Acts 167 and 68 of 2025; Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application (2025); AHS Transformation Reports (2025).

IV. The Six Diagnostic Questions

Each pillar answers a specific diagnostic question in the transformation logic. These questions are not rhetorical. They are the tests that every reform intervention must pass before it is deployed. A policy intervention that passes the Policy question but fails the Technology, Economics, or Operations questions is not a solution — it is a well-intentioned mistake waiting to be made.

Pillar	Diagnostic Question	What It Tests	Failure Mode When the Answer Is No
Policy	Is it permissible and mandatory?	Whether the legal and regulatory architecture makes the intervention binding, not merely aspirational. Whether high-cost actors can opt out.	Voluntary reform. The highest-cost actors decline to participate. The intervention achieves results only among willing participants and leaves the cost structure unchanged.
Technology	Is it visible?	Whether the data infrastructure exists to measure what the intervention is supposed to manage — attribution, TCOC, risk stratification, equity stratification, quality gaps.	'Managing blind.' Financial accountability is real; management capability is absent. Value-based contracts produce unexpected losses because the organization cannot see what it is managing.
Economics	Is it financially rational?	Whether the payment model creates a financial incentive for the behavior the intervention requires — specifically, whether population health management is rewarded rather than penalized.	Clinical redesign that reverts. Programs that demonstrate outcomes but cannot capture the financial return because the payment system still rewards volume. Investment without return.
Clinical	Is it effective?	Whether the care delivery models, workforce design, and clinical programs can actually produce the health improvement and utilization reduction the payment model's economics require.	Financial pressure without clinical pathway. Global budgets create the incentive to reduce hospitalizations; without clinical programs that can actually reduce them, the budget produces access restriction rather than population health improvement.
Equity	Is it just — and designed to be?	Whether the intervention reduces or exacerbates disparities by race, income, and geography — and whether equity requirements are embedded in the design, not reviewed at the end.	Technically successful, ethically failed transformation. Average outcomes improve; disparities widen. Payment design that does not adjust for social risk penalizes safety-net providers. Transformation that reaches the already well-served.
Operations	Is it executable?	Whether the organizational capacity, management	The document graveyard. Statutory mandates exist on

		infrastructure, and implementation discipline exist to translate strategy into functioning programs with trained staff and measured outcomes.	paper. Clinical programs are designed. Data platforms are procured. Nothing is implemented at scale because the organization lacks the capacity to execute on its statutory timeline.
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Figure 1.2 — The six diagnostic questions: the tests every reform intervention must pass. Source: six-pillar framework documentation; CMMI evaluation literature (2023); Vermont experience.

V. The Portfolio Structure — How the Pillars Relate

The six pillars are the portfolio's asset classes. But unlike a financial portfolio where assets can in principle be combined in any proportion, the six pillars have hard structural relationships that constrain how they can be combined and in what order they must be activated. Three types of relationship organize the fifteen directed connections between the pillars:

Relationship Type	What It Means	Critical Example
Enables (critical path)	The upstream pillar's presence is required for the downstream pillar to perform its core function. If the enabling pillar is absent or underdeveloped, the downstream pillar's core function is disabled — not merely weakened.	Policy → Economics: mandatory authority enables structural payment reform. Without the Policy pillar's mandatory architecture, the Economics pillar's payment reform remains voluntary and the highest-cost actors opt out. Vermont's decade of OneCare experience is the proof case.
Requires	The downstream pillar's management function depends on a specific output the upstream pillar must produce. The downstream pillar can be designed without the upstream output, but it cannot be managed without it.	Technology → Economics: VBC contracts require data infrastructure. A hospital can sign a global budget agreement without analytics capability. It cannot manage its performance under that agreement, dispute errors in TCOC calculations, or identify where costs exceed benchmark.
Constrains	The downstream pillar imposes design requirements on the upstream pillar that must be built in from the beginning, not retrofitted afterward. Constraining relationships run against the primary direction of authority in the framework.	Equity → Economics: equity constrains payment design. Global budgets set without social risk adjustment systematically penalize hospitals serving high-SDOH populations. Vermont's Northeast Kingdom hospitals would be penalized for social disadvantage, not operational inefficiency, if the AHEAD global budget methodology ignores their population's underlying risk profile.

Figure 1.3 — Three types of inter-pillar relationship. The complete architecture of all fifteen directed dependency relationships is developed in Chapter 1A.

These three relationship types generate specific, testable predictions about what fails when any pillar is absent, underfunded, or activated in the wrong sequence. Chapter 1A develops all fifteen relationships with their

mechanisms, failure modes, and Vermont evidence. This chapter provides the structural overview: the pillars are a system, the relationships are real and consequential, and the sequencing that follows from the dependency structure is the subject of Chapter 2.

Why the Dependency Structure Matters for Real Decisions

The dependency architecture is not an academic diagram. It has three direct management applications that every reform architect and health system leader must understand before making investment decisions:

SEQUENCING: The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong does not merely slow progress — it produces failed interventions that generate political resistance to the subsequent correct interventions. Chapter 2 develops the full execution sequence. The short version: Policy before Economics. Technology before Economics-as-management. Operations calibrated to a payment environment that is already live.

RISK IDENTIFICATION: The dependency map identifies transformation vulnerabilities precisely. A pillar with many inbound dependencies — one that multiple other pillars require — is the highest-risk gap in the architecture. If it fails or is delayed, multiple other pillars are simultaneously impaired. In Vermont's current architecture, the Technology pillar's analytics platform has the highest inbound dependency count: Economics requires it for AHEAD global budget management, Equity requires it for demographic stratification, and Clinical requires it for population health risk stratification. A delay in analytics vendor deployment does not merely delay one program — it delays three pillar functions simultaneously.

INVESTMENT PRIORITIZATION: Not all pillar investments have equal leverage in the dependency structure. An investment in a pillar with many inbound dependencies has higher system-level leverage than an investment in a pillar that is required by only one other. This is the portfolio management insight that single-pillar thinking consistently misses: the highest-return investment is not necessarily the investment in the pillar whose individual ROI looks largest in isolation. It is the investment in the pillar whose absence impairs the most other pillars.

VI. The Execution Sequence — What Chapter 2 Develops

The dependency structure of the six pillars generates a specific, defensible execution sequence. Chapter 2 develops that sequence in full, with OneCare Vermont's failure as the primary negative proof case and Vermont's Blueprint for Health as the primary positive proof case. The sequence is introduced here because it is fundamental to understanding what the six-pillar framework is — a portfolio with a sequencing discipline, not a simultaneous six-way push.

Stage	Pillar	Core Logic	Vermont Anchor
1	Policy	Mandatory architecture and transformation capital must precede all else. Without statutory mandate, the highest-cost actors opt out. Without federal capital secured through policy relationships, the technology build cannot be funded.	Acts 167, 51, 68; AHEAD State Agreement; \$195M RHT Program award; GMCB mandatory rate authority

2	Technology	Data infrastructure must be built before financial risk is assumed. Economics-as-design (payment architecture definition) proceeds in parallel. Economics-as-management (running global budgets) waits until the technology substrate is operational.	VHCURES; VITL/VHIE; AHS-GMCB analytics vendor; Vermont CIN; AI scribe grants
3	Economics	Payment incentives deploy on a system that can already see itself. Global budgets and reference-based pricing go live. Equity's social risk adjustment requirement must be embedded in payment design at this stage, not retrofitted later.	Mandatory RBP (FY2027); global hospital budgets (FY2028-2030); AHEAD TCOC accountability; EAST Fund
4	Clinical	Care redesign on aligned incentives. This is where the economic return on the entire portfolio is realized. Without Stages 1-3 already in place, clinical transformation operates on misaligned incentives and reverts.	Blueprint for Health (5.8:1 ROI); CoCM; CCBHC expansion; primary care AHEAD; COE regionalization
5	Equity	Systematic measurement of whether Stages 3-4's equity commitments have produced equitable outcomes — and corrective action informed by real population data from a functioning system.	HRSN screening at scale; EAST Fund for SDOH; social risk adjustment audit; REH model; NE Kingdom focus
6	Operations	Closing the administrative cost gap and translating strategy into implemented programs. Operational redesign calibrated to the payment environment now live. Vermont's \$1,303/discharge administrative gap is the quantified opportunity.	RHRC methodology; 14-hospital transformation planning; Vermont CIN shared services; PMO establishment

Figure 1.4 — The six-stage execution sequence: summary. Full development with Vermont evidence, failure cascade analysis, and the OneCare sequencing autopsy in Chapter 2. Source: HTR Analysis (2026); Vermont experience 2010-2026.

VII. The Vermont Thread — Why One State Runs Through Every Chapter

Vermont appears in this book in four registers, and knowing which register you are reading helps calibrate how to use the material.

Vermont as Diagnostic Case

Oliver Wyman's finding that nine of fourteen Vermont hospitals reported operating losses in FY2023, with thirteen of fourteen projected to report losses by 2028, is not a Vermont peculiarity. It is an instance of a generalizable pattern: demographic aging transferring patients from commercial insurance to below-cost Medicare and Medicaid reimbursement; fee-for-service payment rewarding volume in a system that needs to reduce it; voluntary payment reform allowing high-cost actors to opt out. Vermont faces these pressures earlier and more acutely. Every state will face them. Vermont as diagnostic case teaches the pattern, not just Vermont's version of it.

Vermont as Legislative Laboratory

Acts 167, 51, 68, and 62 are specific legislative interventions in a specific state. Their design logic — the reform cascade structure, the mandatory versus voluntary distinction, the statutory accountability metrics, the creation of enabling institutions before deploying payment reform — is generalizable to any state undertaking comparable reform. Vermont as legislative laboratory teaches the policy architecture, not just the Vermont statutes.

Vermont as Operational Field Report

The 148 individual transformation initiatives documented across Vermont's fourteen hospital plans in January 2026, the AHS monthly transformation reports, and the RHRC engagement methodology are real-time accounts of a transformation in progress. They convey the texture of execution — the difficulties, the adaptations, the gap between strategy and implementation — that no policy analysis can substitute for. Vermont as operational field report teaches what execution actually looks like.

Vermont as National Template

Chapter 16 (AHS Restructuring) and Chapter 15 (The Future) use Vermont explicitly as a template for the national condition. The organizational design questions Vermont is answering — how does a state agency restructure from program administration to population health system operation; how do you build analytics capacity when it does not currently exist; how do you manage a multi-stakeholder strategic planning process without producing a lowest-common-denominator plan — are questions every state will face. Vermont as national template teaches the transferable framework.

VIII. How This Book Is Structured Around the Framework

The six-pillar framework organizes every chapter that follows. Each pillar has a dedicated chapter pair: a conceptual chapter that develops the pillar's theory, evidence base, and national context, and a Vermont application chapter that shows the pillar working — or failing — in real time. Chapter 1A develops the full dependency architecture. Chapter 2 develops the execution sequence. The following table provides the navigational map.

Chapter(s)	Pillar / Topic	Core Question Answered
1	The Six-Pillar Framework	What are the six pillars, what function does each perform, and why must all six move together?
1A	The Dependency Architecture	What are the fifteen directed dependency relationships between the pillars, how do they work, and what breaks when any dependency is violated?
2	The Execution Sequence	In what order must the pillars be activated, why does that order follow from the dependency structure, and what does sequencing failure look like in practice?
3-4	Policy Pillar	What legislative and regulatory architecture is required to convert voluntary reform into mandatory structural change, and how does Vermont's reform cascade demonstrate it?
5-6	Technology Pillar	What data infrastructure is required before financial risk can be assumed, and what is the 'managing blind' failure mode that results when Economics precedes Technology?
7-8	Economics Pillar	How do reference-based pricing and global budgets restructure the financial logic of the hospital system, and what financial modeling does VBC readiness require?
9-10	Clinical Pillar	How does clinical redesign convert payment incentives into population health outcomes, and what does the 5.8:1 ROI from Vermont's Blueprint for Health tell us about what works?

11-12	Equity Pillar	How must equity constrain every preceding pillar's design, and how is the equity audit in Stage 5 different from equity review bolted onto a finished system?
13-14	Operations Pillar	How does the Operations pillar translate strategy into implemented programs, and what does Vermont's \$1,303/discharge administrative cost gap tell us about the opportunity?
15	The National Forecast	On what timeline do other states arrive at Vermont's current conditions, and what does the transferable framework look like in systems with different starting conditions?
16	AHS Restructuring Roadmap	How must a state agency restructure itself from program administrator to population health system operator, and what is the organizational design that Act 68 requires?
App. A	Vermont System Portrait	Complete factual foundation: demographics, hospital system, insurance market, workforce, and transformation status as of early 2026.
App. B-D	Reference Materials	Act 68 statutory timeline; Centers of Excellence assignment framework; AHEAD State Agreement key terms.

Figure 1.5 — Book structure mapped to the six-pillar framework. Source: The Health Reform Portfolio framework documentation.

IX. The Master Reference Table — The Framework at a Glance

This section is the reference that readers return to throughout the book. Every subsequent chapter assumes familiarity with the definitions, diagnostic questions, and structural roles summarized here. When the text of any chapter refers to 'the Policy pillar's mandatory architecture' or 'the Technology pillar's data substrate' or 'the Equity pillar's constraining dependency,' this table is the source of those definitions.

Pillar	Diagnostic Question	Structural Role	What It Produces When Functioning	Vermont Anchor
Policy	Is it permissible and mandatory?	The mandatory architecture. Converts aspirational reform into binding requirements. Secures transformation capital.	Statutory mandates eliminating voluntary opt-out; accountability timelines that cannot be deferred; federal-state agreements committing both parties to specific outcomes; transformation grant capital.	Acts 167, 51, 68; AHEAD State Agreement; \$195M RHT award; GMCB mandatory rate authority
Technology	Is it visible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and	Attribution lists; TCOC measurement; HEDIS stratification; demographic risk stratification; care gap identification;	VHCURES; VTTL/VHIE; AHS-GMCB analytics vendor; Vermont CIN; AI scribe grants

		strategic planning analytically feasible.	AI-assisted clinical workflows; real-time population health dashboards.	
Economics	Is it financially rational?	The incentive architecture. Creates the financial logic that makes population health management rational rather than penalized.	Payment models in which prevention is financially rewarded; price structures breaking the commercial cross-subsidy chain; global budgets making volume expansion irrational; total cost of care accountability.	RBP mandatory FY2027; global budgets FY2028-2030; AHEAD TCOC accountability; EAST Fund up to \$150M/year
Clinical	Is it effective?	The mechanism of change. Converts payment incentives into actual health outcomes and utilization reduction.	Primary care transformation preventing hospitalizations; behavioral health integration reducing crisis presentations; care management closing gaps before they become acute episodes; workforce deployed at top of licensure.	Blueprint for Health (5.8:1 ROI); CoCM; CCBHC expansion (7 entities 2026); primary care AHEAD; COE regionalization
Equity	Is it just — and designed to be?	The cross-cutting constraint and accountability lens. Applied to every other pillar's design from the outset, not reviewed at the end.	Stratified measurement revealing disparities hidden in aggregates; payment designs not penalizing safety-net providers; clinical programs reaching disadvantaged populations; social risk adjustment embedded in global budgets.	HSA-level geographic monitoring; HRSN screening; NE Kingdom focus; REH model; Health Equity Advisory Commission role in Strategic Plan
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality.	Hospital transformation plans that are implemented, not filed; AHS organizational capacity sufficient to manage 14 simultaneous transformation processes; PMO tracking progress against statutory deadlines; administrative cost reduction.	RHRC methodology; 148 hospital plan initiatives (Jan 2026); Vermont CIN shared services; AHS HSA Coordinators; \$1,303/discharge admin gap to close

Figure 1.6 — The Six-Pillar Master Reference Table. This is the framework anchor for every subsequent chapter. Diagnostic questions, structural roles, and Vermont anchors reflect the framework as of April 2026. Sources: six-pillar framework documentation; Acts 167 and 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); AHS Health Care System Transformation Reports (2025); GMCB records.

Key Concepts Introduced in This Chapter

Term	Definition
The Health Reform Portfolio	The organizing metaphor and title of this book. Healthcare transformation understood as a portfolio of six interdependent investments — Policy, Technology, Economics, Clinical, Equity, Operations — managed together with explicit attention to sequencing, dependencies, and risk, rather than as a sequential plan or a checklist of independent reforms.
Six-pillar framework	The analytical architecture organizing healthcare transformation across six interdependent domains. Transformation requires all six pillars to be functional; failure in any one pillar undermines the others through a cascade of dependency failures.
Single-pillar thinking	The dominant failure mode of American healthcare reform: addressing one pillar — typically payment reform (Economics) or clinical redesign (Clinical) — while leaving the other five pillars unchanged. Each isolated intervention runs into the constraints created by the pillars it ignores.
Enables (critical path)	A dependency relationship in which the upstream pillar's presence is required for the downstream pillar to perform its core function. The failure of an enabling pillar immediately disables the downstream pillar's core function. Policy → Economics (mandatory authority enables structural payment reform) is the framework's most critical enabling dependency.
Requires	A dependency relationship in which the downstream pillar's management function depends on a specific output the upstream pillar must produce. The downstream pillar can exist without the upstream output but cannot be managed without it. Technology → Economics (data infrastructure required for VBC contract management) is the framework's most consequential 'requires' dependency — and the one that OneCare Vermont violated.
Constrains	A dependency relationship in which the downstream pillar imposes design requirements on the upstream pillar from the outset. Equity → Economics (equity constrains payment design through social risk adjustment) and Equity → Clinical (equity constrains care delivery design through cultural competence requirements) are constraining dependencies.
Reform cascade	A legislatively structured sequence in which each reform intervention builds on the institutional capacity and political legitimacy created by its predecessor. Vermont's Act 167 (2022) → Act 51 (2023) → Act 68 (2025) is the prototype.
Structural transformation	Change that alters the fundamental incentives, relationships, and capabilities of a system — as distinguished from incremental improvement within the existing

	structure. Voluntary ACO programs are not structural transformation. Mandatory global budgets enforced by a state regulatory body are.
Economics-as-design vs. Economics-as-management	A critical internal distinction within the Economics pillar. Economics-as-design (defining the payment architecture: units of accountability, risk allocation, performance metrics) can proceed in parallel with the Technology build. Economics-as-management (running global budgets operationally: monitoring performance, calculating savings, holding providers accountable) requires the Technology substrate to already be functional. OneCare Vermont's failure was the result of deploying Economics-as-management before Economics-as-design and Technology were both in place.
Vermont as preview	The analytical premise that Vermont's healthcare challenges — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary reform failure — are present nationally on a ten-to-twenty-year lag, making Vermont's responses a template for the nation's future policy toolkit.

Key Concepts — Chapter 1. Full glossary of all terms across the book available in the Index.

Sources

six-pillar framework documentation; Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025); Vermont Agency of Human Services Health Care System Transformation Reports (August 2025, November 2025); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); Vermont AHEAD State Agreement, Cohort 2 (January 17, 2025); GMCB Hospital Budget Review records (FY2024, FY2025, FY2026); CMMI model evaluation literature, including Congressional Budget Office score (2023); Maryland HSCRC performance data; Vermont Blueprint for Health Annual Report and ROI study (Population Health Management); AHS Hospital Transformation Plans Analysis (January 2026); Vermont Department of Health Health Equity Data Report (January 2025); all sources cited in Chapter 1A and Chapter 2.